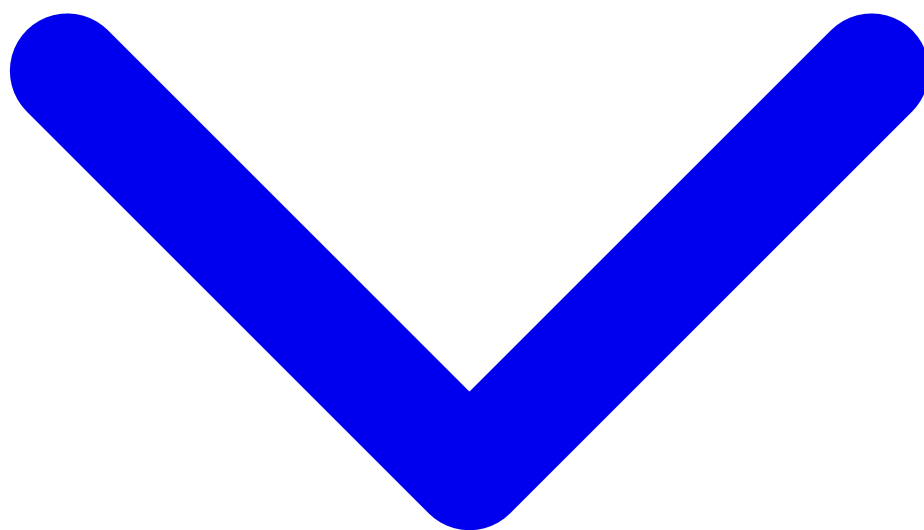


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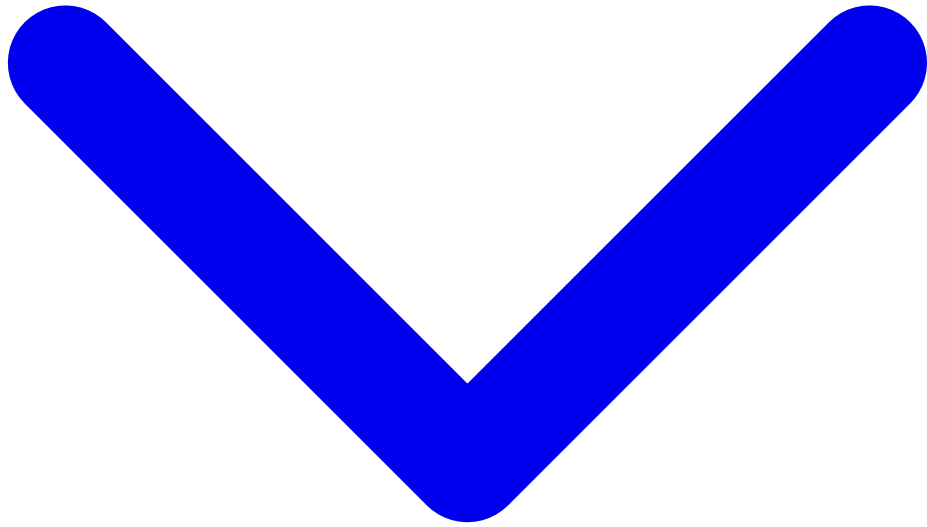
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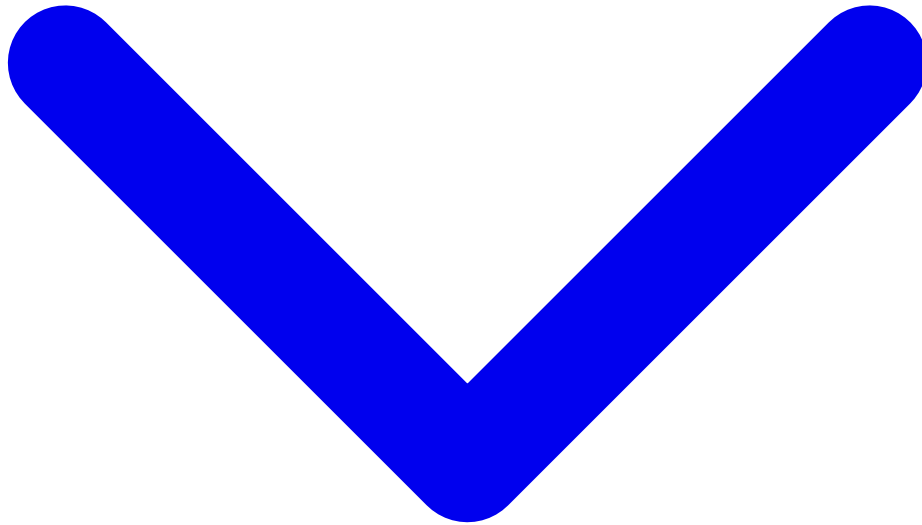
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Developmental Screening Tool

Autism & Social Communication Observation Guide

Organize concrete behavioural observations across home and community settings before clinical assessment.



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Autism & Social Communication Observation Log

Framework: DSM-5-TR Diagnostic Criteria & CDC Developmental Surveillance Guidelines

www.rabbiapsychologist.com

Clinical Parent Resource

Child's Name:

Date of Birth / Age:

Observation Window:

Primary Concern:

DOMAIN A: Social-Emotional Reciprocity & Communication Patterns

Check items observed regularly and note specific examples below:

☐ **Response to Name:** Does not consistently turn head or make eye contact when called from across the room (hearing verified intact). ☐ **Joint Attention & Pointing:** Rarely points to show interest ("Look at that plane!") or follows another person's pointing finger. ☐ **Conversational Turn-Taking:** Struggles with two-way conversation; may monologue extensively on preferred topics or repeat phrases (echolalia). ☐ **Peer Play Differences:** Prefers solitary play or parallel play; difficulty understanding rules of reciprocal games or imaginary play.

Specific Observations / Examples:

DOMAIN B: Restricted, Repetitive Patterns & Sensory Sensitivities

☐ **Insistence on Sameness / Transitions:** Extreme distress over minor route changes, clothing tags, food textures, or routine shifts. ☐ **Repetitive Motor Movements:** Hand flapping, rocking, toe-walking, spinning objects, or lining up toys instead of functional play. ☐ **Sensory Hyper-reactivity:** Covering ears during blenders/hand dryers; extreme sensitivity to bright fluorescent lights or specific clothing fabrics. ☐ **Sensory Hypo-reactivity / Seeking:** High pain tolerance; seeking intense deep pressure, spinning, or smelling/licking non-food objects.

Specific Observations / Examples:

Clinical Notice: This checklist is an observational recording sheet, not a diagnostic instrument. An Autism Spectrum diagnosis requires comprehensive standardized clinical assessment (e.g. ADOS-2, ADI-R) by a qualified developmental specialist.

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ADHD & Child Development Support at Home — Care Beyond the Session

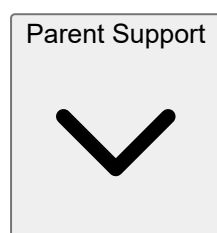
Clinically led by Rabbia Ashraf, Clinical Psychologist.

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